

1. Administrative & Study Identification

Full Study Title: A Phase 3, Randomized, Double-blind, Placebo-controlled Study of BION-1301 in Adults With IgA Nephropathy
Short Title/Acronym: The BEYOND Study **Protocol Number:** CFUB523A12301 / CHK02-02 **NCT Number:** NCT05852938 **Sponsor Name:** Novartis Pharmaceuticals (originally Chinook Therapeutics)
Investigational Product: Zigakibart (BION-1301) / Placebo **Phase of Development:** Phase 3 **Medical Monitor:** Novartis Clinical Operations

2. Study Synopsis & Rationale

2.1 Study Objective Primary Objective: To evaluate the effect of zigakibart (BION-1301) versus placebo on proteinuria (measured by the change in urine protein-to-creatinine ratio [UPCR] from baseline to Week 36) and on long-term estimated glomerular filtration rate (eGFR). **Secondary Objectives:** To evaluate the effect of zigakibart versus placebo on long-term kidney function (change in eGFR from baseline to Week 104), overall safety, and tolerability.

2.2 Background & Rationale IgA nephropathy (IgAN) is the leading cause of primary glomerulonephritis and has limited treatment options. Zigakibart is a novel, humanized monoclonal antibody that blocks APRIL (a proliferation-inducing ligand), a cytokine that is elevated in patients with IgAN and promotes the production of pathogenic galactose-deficient IgA1 (Gd-IgA1). Blocking APRIL with zigakibart is a potential disease-modifying approach designed to reduce inflammation, minimize kidney injury, and prevent progressive kidney function loss.

3. Study Design & Methodology

3.1 Design Framework Study Type: Interventional **Control Type:** Placebo-controlled **Blinding:** Double-blind **Randomization:** 1:1 (Stratified by region, baseline proteinuria, and eGFR)

3.2 Planned Enrollment & Duration Target \$N\$: Approximately 350 participants (330 in the primary cohort, plus 20 in an exploratory cohort with lower eGFR). **Number of Sites:** ~201 global locations **Estimated Study Start:** 06-Jul-2023 **Estimated Primary Completion:** 19-Jan-2026

4. Subject Selection (Eligibility Criteria)

4.1 Inclusion Criteria 1. Male and female participants aged ≥ 18 years. 2. Biopsy-proven IgAN diagnosed within the past 10 years prior to Screening. 3. $eGFR \geq 30$ mL/min/1.73m² and total urine protein ≥ 1.0 g/day (or UPCR ≥ 0.7 g/g). 4. Stable on a maximally tolerated dose of angiotensin-converting enzyme inhibitors (ACEi) and/or angiotensin II receptor blockers (ARB) for at least 12 weeks prior to Screening.

4.2 Exclusion Criteria 1. Secondary forms of IgAN, diagnosis of IgA Vasculitis, or current/history of nephrotic syndrome. 2. Average blood pressure $> 150/90$ mm Hg (systolic/diastolic) at Screening. 3. Current severe infection requiring antimicrobials, or positive serology tests for active HAV, HBV, HCV, or HIV.

5. Intervention & Treatment Plan

5.1 Investigational Regimen Dosage/Frequency: 600 mg every 2 weeks (Q2W). **Route of Administration:** Subcutaneous (SC) injection. **Duration of Treatment:** 104 weeks (followed by a 24-week safety follow-up or enrollment in an optional open-label extension).

5.2 Concomitant & Prohibited Therapy Permitted: Stable and well-tolerated doses of ACEi, ARB, sodium glucose cotransporter-2 inhibitors (SGLT2i), endothelin receptor antagonists (ERAs), and/or mineralocorticoid receptor antagonists (MRAs). **Prohibited:** Systemic immunosuppressant medications and live vaccinations within specified protocol windows.

6. Study Timeline & Visit Schedule

Visit ID	Window (Days)	Key Activities/Procedures
Screening	Day -42 to 0	Informed Consent, Medical/Biopsy History, 24-hour urine collection, eGFR Labs
Baseline	Day 0	Randomization, First SC Dose, Baseline UPCR/eGFR assessments
Interim	Weeks 2 to 104	Q2W dosing. Primary efficacy assessment at Week 36 (UPCR). Safety labs, eGFR monitoring.
End of Study	Week 128	Final eGFR/UPCR (Week 104 for treatment), plus 24-week off-treatment safety follow-up (if not entering Open-Label Extension).

7. Outcome Measures (Endpoints)

7.1 Primary Endpoint Definition: Change in urine protein-to-creatinine ratio (UPCR) from baseline to Week 36, and change from baseline in eGFR. **Measurement Method:** Central laboratory analysis of 24-hour urine collections and serum creatinine levels.

7.2 Secondary & Exploratory Endpoints * Change in eGFR from baseline to Week 104. * Safety and tolerability profile (incidence of treatment-emergent adverse events).

8. Safety & Adverse Event Reporting

Adverse Event (AE) Monitoring: Standard collection via onsite visits and telemedicine/home-health remote visits. **Serious Adverse Events (SAEs):** Standard reporting timeline (typically 24 hours to the Sponsor). **Data Monitoring Committee (DMC):** Independent safety monitoring mechanisms managed by Novartis.

9. Statistical Analysis Plan (SAP) Summary

Analysis Populations: Intent-to-Treat (ITT) for primary efficacy outcomes for the primary cohort ($eGFR \geq 30$). The exploratory cohort ($eGFR \geq 20$ to < 30) will be analyzed separately. **Sample Size Justification:** $N \approx 330$ for the primary cohort powered to detect a statistically significant difference in UPCR reduction and eGFR stabilization between the zigakibart and placebo arms. **Handling of Missing Data:** Standard imputation methods (e.g., Multiple Imputation or Mixed-Model Repeated Measures) for continuous laboratory endpoints like eGFR and UPCR.

10. Quality Assurance & Regulatory Compliance

GCP Compliance: This study will be conducted in accordance with Good Clinical Practice (GCP) and the Declaration of Helsinki. **Monitoring Plan:** Routine onsite monitoring visits. To facilitate study participation over 2.5 years, remote study visits using telemedicine and home health are offered. **Audit Trail:** Standard clinical data management including eCRF locking and central laboratory data reconciliation.

11. Study Identifiers & Governance

Primary ID: CFUB523A12301 / **CHK02-02** **Registry Number:** NCT05852938 **Sponsor/Lead Organization:** Novartis Pharmaceuticals **Study Title:** The BEYOND Study **Official Regulatory Title:** A Phase 3, Randomized, Double-blind, Placebo-controlled Study of BION-1301 in Adults With IgA Nephropathy

12. Clinical Framework (IgA Nephropathy Specific)

Primary Condition: IgA Nephropathy (IgAN) **Diagnosis Threshold:** $eGFR \geq 30$ mL/min/1.73m² and UPCR ≥ 0.7 g/g **Medical Methodology:** Anti-APRIL monoclonal antibody (Zigakibart) +

Guideline-directed RASi Therapy **Optimization Target:** Significant proteinuria (UPCR) reduction and long-term eGFR stabilization

13. Study Procedures & Methodology

Management Pathway: Standard clinical pathway for IgAN + investigational SC administration **Education Protocol (HCP):** Protocol-specific training for remote monitoring and SC injection oversight **Education Protocol (Patient):** Onsite training to permit self-administration of the study drug at home **Quality Audit Mechanism:** Centralized laboratory assessments and routine eCRF clinical site monitoring

14. Observation & Follow-Up Schedule

Total Duration: 128 weeks (104 weeks of double-blind treatment + 24 weeks of safety follow-up) **Onsite Visit Cadence:** Baseline, Week 36, Week 104, and End of Study (Week 128) **Remote Monitoring Frequency:** Telemedicine and home health options available for interim dosing visits **Checklist Review Interval:** Every 2 weeks (Q2W) in alignment with dosing

15. Sign-off & Approval

Role	Name	Signature	Date	:--	:--	:--	:--
Principal Investigator	[Name]	_____	[DD-MMM-YYYY]				
Clinical Operations Lead	[Name]	_____	[DD-MMM-YYYY]		Lead		
Statistician	[Name]	_____	[DD-MMM-YYYY]				